Clopidogrel

Mechanism of action

- Clopidogrel has antiplatelet activity.
- Clopidogrel antagonises (blocks) the binding of adenosine diphosphate (ADP) to platelets and impairs platelet function. Clopidogrel provides significantly more antiplatelet activity than aspirin.

Authorised practice levels

- Paramedics, ICPs and CCPs.

Indications

- ✓ STEMI in conjunction with fibrinolytic therapy.

Contraindications

- ✕ Known severe allergy.

Cautions

- ⚠ Clinically significant bleeding. Clopidogrel will increase bleeding.
- ⚠ At risk of bleeding. If there are any cautions present within the fibrinolytic therapy checklist, personnel must seek clinical advice.
- ⚠ Pregnancy.

Use in pregnancy or when breastfeeding

- Safety has not been demonstrated during pregnancy. The likelihood of STEMI occurring in a woman who is pregnant is so low that personnel must seek clinical advice.
- May be administered if the patient is breastfeeding. Advise the patient to stop breastfeeding and seek further advice from their lead maternity carer or GP.

Dosage

- 300 mg if the patient is aged less than 75 years.

Clopidogrel

- 75 mg if the patient is aged greater than or equal to 75 years.

Administration

- Administer PO.

Common adverse effects

- Increased bleeding.

Usual onset of effect

- 30-60 minutes.

Usual duration of effect

- 3-5 days. This is because platelets exposed to clopidogrel are impaired for the life of the platelet which is 7-10 days. Approximately 10% of platelets are replaced each day.

Usual preparation

- 75 mg tablets.

Pharmacokinetics

- Clopidogrel is a prodrug and must be metabolised to the active form in the liver.
- There are no significant effects from liver impairment on acute administration.

Common interactions

- The risk of bleeding will be increased if the patient is taking an anticoagulant.

More Information

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Version history

11 September 2023 1.0.4

Update: Cautions updated.